STAFF INFO
Name: Lara, Toni RN
Patient INFO
Name: PHILLIPS, PRISCILA
DOB: 1953-08-31

OASIS Assessment: OASIS E1 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number
037834

(M0014) Branch State
Branch State: **Arizona**

(M0016) Branch ID Number

(M0018) Attending Physician (NPI)
1548688120

Physician:
WONG, JOSEPH

Address:
1515 W CHANDLER BLVD CHANDLER AZ 85224

☐ **Not Applicable**

(M0020) Patient ID number
143

(M0030) Start of Care Date
03/24/2026

(M0032) Resumption of Care Date
mm/dd/yyyy

☑ **Not Applicable**

(M0040) Patient Name

First Name:	Middle Name:	Last Name:	Suffix:
PRISCILA		PHILLIPS	

(M0050) Patient State of Residence
State of Residence: **Arizona**

(M0060) Patient Zip Code
85019

(M0063) Medicare Number
5TJ7ER0JU61

☐ **Not Applicable**

(M0065) Medicaid Number

☑ **Not Applicable**

(M0064) Social Security Number

☑ **Not Applicable**

(M0066) Birth Date *
08/31/1953

(M0069) Gender *
Gender: **Female**

(M0080) Discipline Completing Assessment:
◉ 1. RN ○ 2. PT ○ 3. SLP/ST ○ 4. OT

Assigned Staff:
Lara, Toni RN

(M0090) Date Assessment Completed:
03/24/2026

(M0100) Assessment Reason
◉ 1. Start of Care

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply

- ☐ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☑ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☐ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☑ Z. None of the above

(A1110) Language

A. What is your preferred language?
English

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
◉ 0 - No ○ 1 - Yes ○ 9 - Unable to determine

(M0102) Date of Physician-ordered Start of Care (Resumption of Care)
If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1250, Transportation, if date entered:
mm/dd/yyyy

☑ **NA - No specific SOC/ROC date ordered by physician**

(M0104) Date of Referral:
Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA:
03/23/2026

(M0150) Current Payment Sources for Home Care: (Mark all that apply.) *

- [ ] 0. None; no charge for current services
- [x] 1. Medicare (Traditional fee-for-service)
- [ ] 2. Medicare (HMO/managed care/Advantage plan)
- [ ] 3. Medicaid (Traditional fee-for-service)
- [ ] 4. Medicaid (HMO/managed care)
- [ ] 5. Worker's compensation
- [ ] 6. Title programs (for example, Title III, V, or XX)
- [ ] 7. Other government (for example, TriCare, VA)
- [ ] 8. Private insurance
- [ ] 9. Private HMO/managed care
- [ ] 10. Self-pay
- [ ] 11. Other (Specify):
- [ ] UK - Unknown

(A1250) Transportation Has lack of transportation kept you from medical appointments, meetings, work, or from getting things needed for daily living?

- [ ] A. Yes, it has kept me from medical appointments or from getting my medications
- [ ] B. Yes, it has kept me from non-medical meetings, appointments, work, or from getting things that I need
- [x] C. No
- [ ] X. Patient unable to respond
- [ ] Y. Patient declines to respond

(M1000) Inpatient Facilities that patient was discharged within the past 14 days? (Mark all that apply.)

- [ ] 1. Long-term nursing facility (NF)
- [ ] 2. Skilled nursing facility (SNF/TCU)
- [ ] 3. Short-stay acute hospital (IPPS)
- [ ] 4. Long-term care hospital (LTCH)
- [x] 5. Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6. Psychiatric hospital or unit
- [ ] 7. Other (specify):
- [ ] NA - Patient was not discharged from an inpatient facility _ **[Go to M1021]**

(M1005) Inpatient Discharge Date (most recent):

03/23/2026

- [ ] UK - Unknown

Primary Reason for Home Health/Medical Necessity:

This is a case of 72-year-old female patient being considered for admission to home health care following her recent hospitalization for persistent dizziness and headaches, which had significantly impacted her ability to perform activities of daily living (ADLs). Her primary diagnosis is vertigo, with secondary diagnoses including essential hypertension, hyperlipidemia, and abdominal aortic ectasia. She has a complex medical history that includes chronic kidney disease, diabetic neuropathy, and obesity. During her hospital stay, she experienced a decline in functional status, requiring moderate assistance with ADLs and ambulation.

The patient's medical status reveals she has been unable to perform basic self-care tasks independently due to her dizziness, which has progressively worsened over the past seven weeks. She has been evaluated by physical therapist, who noted her functional limitations and the need for skilled nursing and therapy to address her safety, mobility, and overall care needs. The patient has demonstrated a fall risk due to her unsteady gait and dizziness, and she requires skilled interventions to manage her medication regimen and monitor her blood pressure, particularly due to her recent initiation of antihypertensive therapy with lisinopril.

Home health services are deemed medically necessary to facilitate her recovery and improve her independence. Skilled nursing care will be required to monitor her vital signs, manage her medications, and provide education on safety precautions to mitigate fall risks. Additionally, physical and occupational therapy will be essential to help her regain her functional independence, improve her strength and balance, and assist in safely performing her ADLs. Given her current level of dependency, which includes requiring assistance with mobility and personal care tasks, as well as the need for regular monitoring of her medical conditions, she qualifies for home health care services under Medicare guidelines. The anticipated length of stay for home health services is projected to be at least a few weeks, depending on her progress.

Homebound Reason

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [x] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [ ] Cane
  - [ ] Manual Wheel Chair
  - [x] Walker
  - [ ] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [ ] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Assistance needed for safe mobility and transfer

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadprilegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [ ] Unable to tolerate standing or walking for extended periods
- [ ] Weakness and pain from recent surgery
- [ ] Experiencing pain impairs/restricts mobility
- [ ] Visually impaired-requires constant attendance/assist

- [x] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [ ] Leaving home is exhausting, extended rest periods to recover
- [ ] Extended respiratory recovery after exertion
- [x] Requires human assist due to unsteady gait/poor balance
- [ ] Cognitively impaired, unable to safely leave home without assist
- [ ] Infectious disease process causing weakness and exhaustion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

High risk for fall and injury

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [ ] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [ ] Leaving home is strenous requiring extended rest to recover
- [x] Fatigue to arrange/use appropriate transportation
- [x] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [ ] Activities restricted due to pain limitations

Other Homebound Reason :

Prognosis

- ( ) 1-Poor
- ( ) 2-Guarded
- (x) 3-Fair
- ( ) 4-Good
- ( ) 5-Excellent

Functional Limitations

- ( ) 1 - Amputation
- ( ) 2 - Bowel/Bladder (Incontinence)
- ( ) 3 - Contracture
- ( ) B- Other (Specify)
- ( ) 4 - Hearing
- ( ) 5 - Paralysis
- (x) 6 - Endurance
- (x) 7 - Ambulation
- ( ) 8 - Speech
- ( ) 9 - Legally blind
- ( ) A - Dyspnea with minimal exertion

Emergency Preparedness:

Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
Notified local utility companies and local emergency offices notified of life supporting equipment being used
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
Discussed patient specific emergency planning options
Educational materials provided to suggest/assist with emergency management/decision making priorities
List of local and state approved evacuation routes and community shelters relevant to the patient's specific geographic location
Obtained the patient's emergency contact numbers for the medical record

Acuity Level:
Medium Risk or Medium Priority

Emergency Triage:
Category 4 - Visit could be postponed 72-96 hours without adverse effects

Additional Emergency Preparedness Information

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [ ] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments:

Advance Directives

Patient has Advance Directives? ( ) Yes (x) No

If you have checked "yes", fill the following fields:

- [ ] Healthcare Representative
- [ ] Do Not Resuscitate
- [ ] Power of Attorney
- [ ] Living Will
- [ ] Conservator Assigned
- [ ] Organ Donor
- [ ] Funeral Arrangements Made
- [x] Full Code — Copies to Follow
- [ ] The patient has not executed any advance directives to health care.
- [ ] The patient was provided with written and verbal information on advance directives.

Comments:

Has surrogate? ( ) Yes (x) No

Surrogate Name:

Surrogate Phone:

Cultural

Primary Language?
- [x] English
- [ ] Spanish
- [ ] Chinese
- [ ] Vietnamese

Other:

Is religion important to the patient? ( ) Yes (x) No

Patient's religious preference:

Patient's primary source of emotional support:
Family and friends

Past Medical History (Mark all that apply)

- [ ] CHF
- [ ] Chest Pain
- [x] HTN
- [ ] Cardiomyopathy
- [ ] MI
- [ ] PVD
- [ ] Arrhythmia
- [ ] CAD
- [ ] Murmur

- [ ] Cancer: (Specify Type): In remission? ◯ Yes ● No
- [ ] Osteoathritis/DJD: (Specify site affected):
- [ ] Joint Replacement: (Specify joint):
- [ ] Rheumatoid Arthritis
- [x] Gait Problems
- [ ] Fractures
- [x] Falls
- [ ] CVA
- [ ] TIA
- [ ] MS
- [ ] Hemiplegia
- [ ] Seizures
- [ ] Headaches
- [x] Dizziness/Vertigo
- [ ] IBS
- [ ] Crohn's Disease
- [x] Diverticulitis/Diverticulosis
- [x] Constipation
- [ ] Diarrhea
- [ ] Fecal Incontinence
- [ ] Liver/Gallbladder Problems: (Specify):
- [ ] Depression
- [ ] Anxiety
- [ ] Dementia
- [ ] Substance Abuse: (Specify):
- [ ] Alzheimer's
- [ ] Mental Disorder: (Specify):
- [ ] Pressure Ulcer
- [ ] Stasis Ulcer
- [ ] Diabetic Ulcer
- [ ] Trauma Wound
- [x] Other: (Specify): Diaphragmatic hernia
- [ ] Chronic Kidney Disease
- [ ] Renal Failure
- [ ] Dialysis
- [ ] Anemia
- [ ] Abnormal Coagulation
- [ ] Blood Clots
- [ ] Diabetes
- [ ] Thyroid Problems
- [ ] COPD
- [ ] Asthma
- [ ] Emphysema
- [ ] Chronic Obstructive Bronchitis
- [ ] Chronic Obstructive Asthma
- [ ] Urinary Incontinence
- [ ] Urinary Retention
- [ ] BPH
- [ ] Recent/Frequent UTI
- [ ] Tuberculosis
- [ ] Hepatitis: (Specify):
- [ ] Infectious Disease: (Specify):
- [ ] Tobacco Dependence

Type: Amount: Length of Time Used:

Past Surgical History:
Denies

Health Screening

Last Cholesterol Level: mm/dd/yyyy

Last Mammogram: mm/dd/yyyy

Does patient perform monthly self breast exam? ◯ Yes ◯ No

Last Pap Smear: mm/dd/yyyy

Last PSA: mm/dd/yyyy

Last Prostate Exam: mm/dd/yyyy

Last Colonoscopy: mm/dd/yyyy

(M1100) Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	◯ Patient Lives alone around the clock	◯ Patient Lives alone Regular daytime	◯ Patient Lives alone Regular Night time	◯ Patient Lives alone Short-term Assistance	◯ Patient Lives alone No Assistance available
B. Patient lives with other person(s) in the home	● Patient lives with other person(s) in the home Around the clock	◯ Patient lives with other person(s) in the home Regular daytime	◯ Patient lives with other person(s) in the home Regular Night time	◯ Patient lives with other person(s) in the home Short-term Assistance	◯ Patient lives with other person(s) in the home No Assistance available
C. Patient lives in congregate situation (for example, assisted living, residential care home)	◯ Patient lives in congregate situation (for example, assisted living, residential care home) around the clock	◯ Regular daytime	◯ Patient lives in congregate situation Regular Night time	◯ Patient lives in congregate situation Short-term Assistance	◯ Patient lives in congregate situation No Assistance available

Caregiver Contact Info:

Primary Caregiver: JOSEPH

Relationship to Patient: son

Contact Number: 480-915-7921

Comments:
Patient lives in one level home with her kids and no steps to enter the home

Name of Facility:

Phone:

Able to safely care for patient? ● Yes ◯ No

ACTIVE DIAGNOSES

(M1021/1023) Diagnoses and Symptom Control

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

OASIS IS ALREADY EXPORTED, PRIMARY DIAGNOSIS AND OTHER DIAGNOSIS FROM 1 TO 5 ARE NOW DISABLED

	Onset Date	Severity						
PRIMARY DIAGNOSIS H81.13 - Benign paroxysmal vertigo, bilateral	03/24/2026	○ 0	○ 1	○ 2	● 3	○ 4	○ ^	○ Onset ● Exacerbate
OTHER DIAGNOSIS - 1 R53.81 - Other malaise	03/24/2026	○ 0	○ 1	○ 2	● 3	○ 4	○ ^	○ Onset ● Exacerbate
OTHER DIAGNOSIS - 2 I10. - Essential (primary) hypertension	03/24/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	○ Onset ● Exacerbate
OTHER DIAGNOSIS - 3 E78.5 - Hyperlipidemia, unspecified	03/24/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	○ Onset ● Exacerbate
OTHER DIAGNOSIS - 4 K57.90 - Dvrtclos of intest, part unsp, w/o perf or abscess w/o bleed	03/24/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	○ Onset ● Exacerbate
OTHER DIAGNOSIS - 5 K44.9 - Diaphragmatic hernia without obstruction or gangrene	03/24/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	○ Onset ● Exacerbate
K59.00 - Constipation, unspecified	03/24/2026	○ 0	○ 1	● 2	○ 3	○ 4	○ ^	○ Onset ● Exacerbate
Z91.81 - History of falling	03/24/2026	○ 0	○ 1	○ 2	○ 3	○ 4	● ^	● Onset ○ Exacerbate

(M1028) Active Diagnoses – Comorbidities and Co-existing Conditions – Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.

- [ ] **1. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)**
- [ ] **2. Diabetes Mellitus (DM)**
- [x] **3. None of the above**

VITAL SIGNS & PAIN ASSESSMENT

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up

A. Height (in inches). Record most recent height measure since the most recent SOC/ROC:
61

B. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.:
183

BMI:
34.57

Comments:

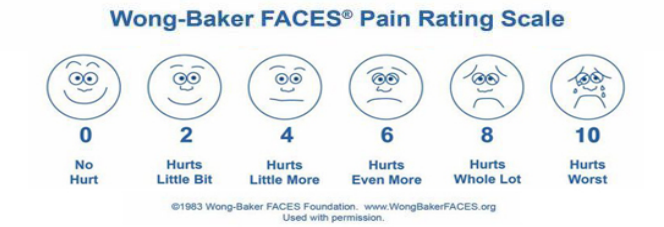
Vital Signs

Temperature: 97.8 Temporal
Respiratory (per minute): 16
Blood Pressure mm/Hg (R): 105 / 67 sitting
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 98
Comments:

Pulse (per minute): Apical = Radial = 72
Weight: 183 reported
Blood Pressure mm/Hg (L): /

O2 Sat in: Room Air

Pain Assessment



0

Collected Using: ☐ Face Scale ☑ Numeric Rating Scale

Pain Location and Description:

Comments:
Patient denies any pain at the time of assessment

(J0510) Pain Effect on Sleep
Ask patient: "Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"

- ○ **0. Does not apply – I have not had any pain or hurting in the past 5 days**
- ● **1. Rarely or not at all**
- ○ **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0520) Pain Interference with Therapy Activities
Ask patient: "Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"

- ○ **0. Does not apply – I have not received rehabilitation therapy in the past 5 days**
- ● **1. Rarely or not at all**
- ○ **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0530) Pain Interference with Day-to-Day Activities
Ask patient: "Over the past 5 days, how often you have limited your day-to-day activities (excluding rehabilitation therapy session) because of pain?"

- ● **1. Rarely or not at all**
- ○ **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost Constantly**
- ○ **8. Unable to answer**

MEDICATION & ALLERGIES (INJECTABLE MEDICATIONS)

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking	2. Indication Noted
	1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	■
E. Anticoagulant	☐	■
F. Antibiotic	☐	■
H. Opioid	☐	■
I. Antiplatelet	☑	☑
J. Hypoglycemic (including insulin)	☐	■
Z. None of the Above	☐	

Medication

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	03/18/2026	Meclizine (Oral Pill)	25 mg 1 Tab As needed	By mouth	ANTIVERTIGO AGENTS / vertigo	Old
☐	03/18/2026	LIPITOR	40 mg 1 Tab At bedtime	By mouth	ANTILIPEMIC AGENTS / Hyperlipidemia	Old
☐	03/18/2026	Bisacodyl (Rectal)	10 mg Suppository As needed	Rectal	LAXATIVES,RECTAL / Bowel Evacuation	New
☐	03/19/2026	Lisinopril (Oral Pill)	5 mg 1 Tab Once a day	By mouth	ACE INHIBITORS / Hypertension	Old

	Start Date		Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	03/24/2026	Aspirin (Chewable)	81 mg 1 Tab Once a day	Chewable	NON-OPIOID ANALGESICS / Myocardial infarction prevention	New
☐	03/24/2026	MIRALAX	17 g 1 packet Once a day as needed	Oral Solution Powder	HYPEROSMOTIC LAXATIVES / Constipation	Old

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

IV

Does patient have IV access? ○ **Yes** ● **No**

(M2001) Drug Regimen Review

Did a complete drug regimen review identify potential clinically significant medication issues?

- ● **0. No – No issues found during review _ [Go to M2010, Patient/Caregiver High-Risk Drug Education]**
- ○ **1. Yes – Issues found during review**
- ○ **Not assessed/no information**
- ○ **9. NA – Patient is not taking any medications _ [Go to O0110, Special Treatments, Procedures, and Programs]**

(M2003) Medication Follow-up

Did the agency contact a physician (or physician-designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

- ○ **0. No**
- ○ **1. Yes**
- ○ **Not assessed/no information**

(M2010) Patient/Caregiver High-Risk Drug Education

Has the patient/caregiver received instruction on special precautions for all high-risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ○ **0. No**
- ● **1. Yes**
- ○ **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

(M2020) Management of Oral Medications

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. (NOTE: This refers to ability, not compliance or willingness.)

- ○ **0. Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ● **1. Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ○ **2. Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ○ **3. Unable to take medication unless administered by another person**
- ○ **NA - No oral medications prescribed**

(M2030) Management of Injectable Medications

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ○ **0. Able to independently take the correct medication(s) and proper dosage(s) at the correct times**
- ○ **1. Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.**
- ○ **2. Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection**
- ○ **3. Unable to take injectable medication unless administered by another person**
- ● **NA - No injectable medications prescribed.**

(O0110) Special Treatments, Procedures, and Programs

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	☐
A3. Oral	☐
A10. Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐
C2. Continuous	☐
C3. Intermittent	☐
C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐

G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐
H10. Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

Most Recent Immunizations

Pneumonia
○ Yes ○ No ● Unknown Date: mm/dd/yyyy

Flu
○ Yes ○ No ● Unknown Date: mm/dd/yyyy

Tetanus
○ Yes ○ No ● Unknown Date: mm/dd/yyyy

TB
○ Yes ○ No ● Unknown Date: mm/dd/yyyy

TB Exposure
○ Yes ● No ○ Unknown Date: mm/dd/yyyy

Hepatitis B
○ Yes ○ No ● Unknown Date: mm/dd/yyyy

Additional Immunizations

Immunization Name:
○ Yes ○ No ○ Unknown Date: mm/dd/yyyy

Immunization Name:
○ Yes ○ No ○ Unknown Date: mm/dd/yyyy

Comments:
Covid 19 vaccination unknown. Patient cannot recall exact date of last immunizations. Taught patient/PCG on the importance of up-to-date immunizations.

Vaccination Record

1st Dose COVID-19
Product Name/Manufacturer (Lot No.): **Date:** mm/dd/yyyy **Healthcare Professional or Clinic Site:**

2nd Dose COVID-19
Product Name/Manufacturer (Lot No.): **Date:** mm/dd/yyyy **Healthcare Professional or Clinic Site:**

Booster 1
Product Name/Manufacturer (Lot No.): **Date:** mm/dd/yyyy **Healthcare Professional or Clinic Site:**

Booster 2
Product Name/Manufacturer (Lot No.): **Date:** mm/dd/yyyy **Healthcare Professional or Clinic Site:**

Allergies

Name	Reaction	Start Date

None known

Add Allergies

NEUROLOGICAL (HEAD, MOOD, EYES, EARS)

Neuro/Emotional/Behavioral Assessment

Conditions of Participation

Neuromascular

Impairment:

- [ ] No Problems identified
- [ ] Speech
- [ ] Hearing
- [ ] Visual
- [ ] Tremors
- [ ] Numbness
- [ ] Tingling
- [x] Vertigo
- [ ] Ataxia
- [ ] Decreased Sensitivity

Others (Specify):

Psychosocial

- [x] WNL (Within Normal Limits)
- [ ] Poor Home Environment
- [ ] Impaired Decision Making
- [ ] Poor Coping Skills
- [ ] Demonstrated/Expressed Anxiety
- [ ] Agitated
- [ ] Inappropriate Behavior
- [ ] Depressed Mood
- [ ] Irritability

- [ ] Document as Clinical Narrative

Educational level

High school

Primary Language

English

Unable to: [ ] *Read* [ ] *Write*

Barriers:

Language

- ( ) Needs interpreter

Learning

- ( ) Mental
- ( ) Physical
- ( ) Psychosocial
- ( ) Functional

Emotions Reported:

- ( ) Angry
- ( ) Discouraged
- ( ) Withdrawn
- ( ) Flat affect
- ( ) Difficulty coping
- ( ) Disorganized

- ( ) Depressed
- ( ) Recent
- ( ) Long Term

Treatment:

Evidence of abuse/neglect/exploitation:

- ( ) Potential
- ( ) Verbal/Emotional
- ( ) Physical
- ( ) Actual
- ( ) Financial
- (●) No Evidence of Abuse/Neglect/Exploitation

Unable to cope with altered health status, patient:

- ( ) Lacks motivation
- ( ) Has unrealistic expectations
- ( ) Denies problems
- ( ) Unable to recognize problems

Sleep/Rest

- (●) Adequate
- ( ) Inadequate

No. of hours slept per night:

0

Comments

Patient states she sleeps for 6- 8 hours a night

Spiritual / Cultural implications that impact care

Spiritual resource

[ ] Inappropriate responses to caregivers/clinician

[ ] Inappropriate follow-through in past

Did patient drive a vehicle before admission? [x] Yes [ ] No [ ] NA

Did the patient have a job before admission? ( ) Yes (●) No

If yes, do they want to drive post-discharge? [x] Yes [ ] No

If yes, do they want to return to work post-discharge? ( ) Yes ( ) No

(B1300) Health Literacy

How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?

- ( ) 0. Never
- ( ) 1. Rarely
- (●) 2. Sometimes
- ( ) 3. Often
- ( ) 4. Always
- ( ) 7. Patient declines to respond
- ( ) 8. Patient unable to respond

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ◯ No (patient is rarely/never understood) ● Yes ◯ Not assessed/no information

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◯ None ◯ One ● Two ◯ Three ◯ Not Assessed

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year
- ◯ 0. Missed by > 5 years or no answer
- ◯ 1. Missed by 2-5 years
- ◯ 2. Missed by 1 year
- ● 3. Correct
- ◯ Not assessed/no information

Ask patient: **"What month are we in right now?"**

B) Able to report correct month
- ● 0. Missed by > 1 month or no answer
- ◯ 1. Missed by 6 days to 1 month
- ◯ 2. Accurate within 5 days
- ◯ Not assessed/no information

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week
- ● 0. Incorrect or no answer
- ◯ 1. Correct
- ◯ Not assessed/no information

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"
- ◯ 0. No – could not recall
- ● 1. Yes, after cueing ("something to wear")
- ◯ 2. Yes, no cue required
- ◯ Not assessed/no information

B) Able to recall "blue"
- ◯ 0. No – could not recall
- ● 1. Yes, after cueing ("a color")
- ◯ 2. Yes, no cue required
- ◯ Not assessed/no information

C) Able to recall "bed"
- ● 0. No – could not recall
- ◯ 1. Yes, after cueing ("a piece of furniture")
- ◯ 2. Yes, no cue required
- ◯ Not assessed/no information

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** 07

Mental Status

- ● 1 - Oriented
- ◯ 2 - Comatose
- ● 3 - Forgetful
- ◯ 4 - Depressed
- ◯ 5 - Disoriented
- ◯ 6 - Lethargic
- ◯ 7 - Agitated
- ◯ 8 - Alert
- ● 9 - Confused
- ◯ 10 - Dementia
- ◯ 11 - Assaultive
- ◯ 12 - Delirious
- ◯ 13 - Wanders
- 14 - Other

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

The patient was well-dressed and appeared freshly showered. During the assessment, she was unable to identify the current month, day, or year, stating she was unsure. She repeated the same question multiple times, indicating possible forgetfulness. Despite this, the patient remained engaged throughout the assessment.

Has there been a sudden/acute change in their mental status? ☐ Yes ☑ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☑ No ☐ Yes

Comments:

(C1310) Signs and Symptoms of Delirium (from CAM©)

A. Acute Onset of Mental Status Change

Is there evidence of an acute change in mental status from the patient's baseline?

● 0. No ◯ 1. Yes

B. Inattention

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- ◯ 0. Behavior not present
- ◯ 1. Behavior continuously present, does not fluctuate
- ● 2. Behavior present, fluctuates (comes and goes, changes in severity)
- ◯ Not assessed/no information

C. Disorganized thinking

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- ◯ 0. Behavior not present
- ● 1. Behavior continuously present, does not fluctuate
- ◯ 2. Behavior present, fluctuates (comes and goes, changes in severity)
- ◯ Not assessed/no information

D. Altered level of consciousness

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- **vigilant** – startled easily to any sound or touch □ lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch
- **stuporous** – very difficult to arouse and keep aroused for the interview
- **comatose** – could not be arouse

- ● 0. Behavior not present
- ◯ 1. Behavior continuously present, does not fluctuate
- ◯ 2. Behavior present, fluctuates (comes and goes, changes in severity)
- ◯ Not assessed/no information

(D0150) Patient Mood Interview (PHQ-2 to 9)

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**

If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**

Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	No ×	Never or 1 day
B. Feeling down, depressed, or hopeless	No ×	Never or 1 day
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^

	Presence	Frequency
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		0

(D0700) Social Isolation

How often do you feel lonely or isolated from those around you?

- [x] 0. Never
- [ ] 1. Rarely
- [ ] 2. Sometimes
- [ ] 3. Often
- [ ] 4. Always
- [ ] 7. Patient declines to respond
- [ ] 8. Patient unable to respond

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed): (Mark all that apply.

- [ ] 1. Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
- [ ] 2. Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
- [ ] 3. Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
- [ ] 4. Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
- [ ] 5. Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)
- [ ] 6. Delusional, hallucinatory, or paranoid behavior
- [x] 7. None of the above behaviors demonstrated

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- [x] 0. Never
- [ ] 1. Less than once a month
- [ ] 2. Once a month
- [ ] 3. Several times each month
- [ ] 4. Several times a week
- [ ] 5. At least daily

(M1700) Cognitive Functioning

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- [ ] 0. Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.
- [x] 1. Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.
- [ ] 2. Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility
- [ ] 3. Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
- [ ] 4. Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- [ ] 0. Never
- [x] 1. In new or complex situations only
- [ ] 2. On awakening or at night only
- [ ] 3. During the day and evening, but not constantly
- [ ] 4. Constantly
- [ ] 5. Patient nonresponsive

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- [x] 0. None of the time
- [ ] 1. Less often than daily
- [ ] 2. Daily, but not constantly
- [ ] 3. All of the time
- [ ] NA - Patient nonresponsive

Eyes and Ears

(B1000) Vision

Ability to see in adequate light (with glasses or other visual appliances)

- [ ] 0. Adequate – sees fine detail, such as regular print in newspapers/books
- [x] 1. Impaired – sees large print, but not regular print in newspapers/books
- [ ] 2. Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- [ ] 3. Highly impaired – object identification in question, but eyes appear to follow objects
- [ ] 4. Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Vision cont.

- [ ] No Problems Identified
- [x] Glasses
- [ ] Macular Degeneration
- [ ] Blurred Vision
- [ ] Glaucoma
- [ ] Contact Lenses:
- [ ] Right
- [ ] Left
- [ ] Cataracts
- [ ] Blindness(Specify):
- [ ] Others (Specify):

Comments:

Pupils

- [ ] No Problems Identified
- [x] PERRLA
- [ ] Anisocoria
- [ ] Right
- [ ] Left
- [ ] Others(Specify):

Comments:

(B0200) Hearing

Ability to hear (with hearing aid or hearing appliances if normally used)

- [x] 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV
- [ ] 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- [ ] 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- [ ] 3. Highly impaired – absence of useful hearing

Hearing cont.

- [x] No Problems Identified
- [ ] Right Hearing Impairment
- [ ] Left Hearing Impairment
- [ ] Deaf
- [ ] Speech Impairment (Specify):

Comments:

CARDIOPULMONARY (CHEST & THORAX)

Heart Sounds

- [x] Normal
- [x] Regular
- [ ] Irregular
- [ ] Murmur
- [ ] Gallops
- [ ] Clicks
- [ ] Friction Rule

Comments (If Abnormal (Explain)):

Edema

Upper:
- [ ] Right Extremities
- [ ] Left Extremities

Lower:
- [ ] Right Extremities
- [ ] Left Extremities

Others:

Comments:

.
- [ ] Cramps
- [ ] Claudication
- [ ] Fatigue

Capillary Refill:
- [x] > 3 secs
- [ ] < 3 secs

PT/INR: /

Peripheral Pulses

Radial:	[x] Strong/Bounding	[ ] Weak	[ ] Thready	[ ] Absent
Pedal:	[x] Strong/Bounding	[ ] Weak	[ ] Thready	[ ] Absent

Comments:

Chest/Thorax

Chest Pain

- [x] Denies
- [ ] Anginal
- [ ] Postural
- [ ] Localized
- [ ] Substernal
- [ ] Sharp
- [ ] Dull
- [ ] Aching
- [ ] Radiating
- [ ] Viselike
- [ ] Stabbing
- [ ] Others (Specify):

Associated with:
- [ ] Shortness of Breath
- [ ] Activity
- [ ] Rest
- [ ] Frequency/Duration:

Comments:

(M1400) When is the patient dyspneic or noticeably Short of Breath?

- (●) 0. Patient is not short of breath
- ( ) 1. When walking more than 20 feet, climbing stairs
- ( ) 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- ( ) 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- ( ) 4. At rest (during day or night)

Respiratory Status

- [ ] Dyspnea:
- [ ] At Rest
- [ ] With Exertion/Activity
- [ ] Accessory Muscle Used
- [ ] Orthopnea
- [ ] Retraction
- [ ] Stridor

O2 lpm O2 Saturation %
- [ ] PRN
- [ ] Continuous
- [ ] Mechanical Ventilator:
- [ ] Intubated
- [ ] Tracheostomy

Others (Specify):

Comments:

Lung Sounds

Anterior: Right: **Clear** Left: **Clear**

Posterior: Upper Right: **Clear** Upper Left: **Clear** Lower Right: **Clear** Lower Left: **Clear**

Comments:

Cough

- [x] None
- [ ] Dry
- [ ] Acute
- [ ] Chronic
- [ ] Non-Productive
- [ ] Productive:
- [ ] Not Assessed/No Information
- [ ] Yes
- [ ] No

Suction:
- [ ] Not Assessed/No Information
- [ ] Yes
- [ ] No
- [ ] Hemoptysis

Frequency: Amount:

Comments:

GASTROINTESTINAL & GENITOURINARY ASSESSMENT

Gastrointestinal (Mouth - Anus)

Mouth and Throat

- [x] No Problems Identified
- [ ] Dentures:
- [ ] Upper
- [ ] Lower
- [ ] Partial
- [ ] Bleeding Gums/Gingivitis
- [ ] Toothache
- [ ] Ulcerations
- [ ] Lesion
- [ ] Dysphagia
- [ ] Hoarseness
- [ ] Sore Throat
- [ ] Masses/Tumors

Comments:

Appetite

- [x] Good
- [ ] Fair
- [ ] Poor

Intake:
- [x] Adequate
- [ ] Inadequate

Abnormalities:
- [ ] Anoxeria
- [ ] Cachexia
- [ ] Nausea
- [ ] Vomiting
- [ ] Decreased Appetite
- [ ] Reflux/Indigestion
- [ ] Diarrhea
- [ ] Constipation
- [ ] Bowel Incontinence
- [ ] Other (Specify):

Comments:

(M1870) Feeding or Eating

Current ability to feed self meals and snacks safely. Note: This refers only to the process of ***eating***, ***chewing***, and ***swallowing***, not preparing the food to be eaten.

- [x] **0. Able to independently feed self.**
- [ ] **1. Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.**
- [ ] **2. Unable to feed self and must be assisted or supervised throughout the meal/snack.**
- [ ] **3. Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.**
- [ ] **4. Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.**
- [ ] **5. Unable to take in nutrients orally or by tube feeding.**

Feeding Cont.

☐ **Formula** ☐ **Continuous Feeding** **cc:** **hours:**

☐ **Bolus:** ☐ **Gravity** ☐ **Pump** **cc:** **every hour(s):**

Flushing Protocol:

PEG/G-Tube Placement:

☐ **Placement Checked** ☐ **Residual Checked** **Amount:**

Comments:

(K0520) Nutritional Approaches

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	☐
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	☐
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	☐
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	☐
Z. None of the above	☑

#16 Nutritional Requirements

☐ NPO ☐ No Added Salt ☐ Low Salt ☐ Low Fat ☐ Low Cholesterol ☐ No Concentrated Sweets

☐ Controlled Carbohydrates ☑ Regular ☐ Fluid Restriction:

Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
○ **Without reason, has lost more than 10 lbs in the last 3 months**	15	○ **Does not always have enough money to buy foods need**	10
○ **Has an illness or condition that made pt change the type and/or amount of food eaten**	10	○ **Eats alone most of the time**	5
○ **Has open decubitus, ulcer, burn or wound**	10		
○ **Eats fewer than 2 meals a day**	10	● **Takes 3 or more prescribed or OTC medications a day**	5
○ **Has a tooth/mouth problem that makes it hard to eat**	10	○ **Is not always physically able to cook and/or feed self and has no caregiver to assist**	5
○ **Has 3 or more drinks of beer, liquor or wine almost every day**	10	○ **Frequently has a diarrhea or constipation**	5
		Total	5

● **Good Nutritional Status (Score 0 - 25)** ○ **Moderate Nutritional Status (Score 25 - 54)** ○ **High Nutritional Status (Score 55 - 100)**

Nutritional Status Commments:

Weight

☑ **No Change** ☐ **Weight Gain** ☐ **Weight Loss**

Abdomen

☑ **Abdominal Soft/Tender** ☐ **Abdominal Distention** ☐ **Girth(cm):**

Comments:

(M1620) Bowel Incontinence Frequency

- [x] **0. Very rarely or never has bowel incontinence**
- [ ] **1. Less than once weekly**
- [ ] **2. One to three times weekly**
- [ ] **3. Four to six times weekly**
- [ ] **4. On a daily basis**
- [ ] **5. More often than once daily**
- [ ] **NA - Patient has ostomy for bowel elimination**
- [ ] **UK - Unknown**

(M1630) Ostomy for Bowel Elimination

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

- [x] **0. Patient does not have an ostomy for bowel elimination**
- [ ] **1. Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.**
- [ ] **2. The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.**

Ostomy/Stoma Appearance

☐ **Pinkished-Red** ☐ **Moist** Abnormalities: ☐ **Purple** ☐ **Black in Color** ☐ **Dry**

☐ **Gray** ☐ **Bad Odor** ☐ **Itches/Hurt** **Other (Specify):**

Bowel Sounds:

Abdominal Quadrants:

Right Lower	Right Upper	Left Lower	Left Upper	Last Bowel Movement:
Normoactive	Normoactive	Normoactive	Normoactive	03/24/2026

Comments:

Constipation

- [ ] Chronic
- [ ] Acute
- [x] Occasional
- [ ] Internal Hemorrhoids
- [ ] External Hemorrhoids
- [ ] Laxative/Enema Use

Comments:

Stool Appearance

- [x] Within Normal Limits
- [ ] Gray
- [ ] Tarry
- [ ] Black
- [ ] Fresh Blood
- [ ] Others (Specify):

Comments:

Genitourinary (Genitals & Urinary)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

- (●) 0. No
- ( ) 1. Yes
- ( ) NA - Patient on prophylactic treatment
- ( ) UK - Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

- ( ) Patient is incontinent
  - ( ) Day time
  - ( ) Night time
  - ( ) Day and Night
  - ( ) Timed-voiding defers incontinence
  - ( ) Occasional stress incontinence
- (●) No incontinence or catheter (includes anuria or ostomy for urinary drainage)
- ( ) Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)

Genitourinary Assessment

- [x] Within Normal Limits

Abnormalities:

- [ ] Frequency
- [ ] Dysuria
- [ ] Urgency
- [ ] Anuria
- [ ] Nocturia
- [ ] Oliguria
- [ ] Retention
- [ ] Hematuria
- [ ] Irrigation
- [ ] Incontinence
- [ ] Diapers Used
- [ ] Penile Discharge
- [ ] Vaginal Bleeding/Discharge

Others (Specify):

Urine Color:

- [ ] Clear
- [ ] Transparent
- [x] Pale Yellow
- [ ] Brownish Orange
- [ ] Pinkish Red
- [ ] Dark Yellow
- [ ] Blue or Green
- [ ] Foamy
- [ ] Transparent Yellow

Urine Consistency:

- [ ] Hematuria
- [ ] Sediment
- [ ] Odorous
- [ ] Cloudy

Urinary Catheter:

- [ ] External Catheter
- [ ] Foley
- [ ] Suprapubic
- [ ] Nephrostomy

Last Catheter Change: mm/dd/yyyy

Fr:

cc:

Comments:

INTEGUMENTARY (SKIN & WOUND)

(M1306) Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?

(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

- (●) 0. No **[→ Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC]**
- ( ) 1. Yes

(M1311) Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non┐removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:

Intact skin with non-blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persistent blue or purple hues.

Stage 1 Pressure Injuries: **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:

(Excludes pressure ulcer/injury that cannot be staged due to a non-removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)

Stage of Most Problematic: **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

- (●) 0. No **[GO TO M1340]**
- ( ) 1. Yes, patient has BOTH observable and unobservable stasis ulcers
- ( ) 2. Yes, patient has observable stasis ulcers ONLY
- ( ) 3. Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non┐removable dressing/device) **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

- ( ) 1. One
- ( ) 2. Two
- ( ) 3. Three
- ( ) 4. Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable

- ◯ 1. Fully granulating
- ◯ 2. Early/partial granulation
- ◯ 3. Not healing

(M1340) Does this patient have a Surgical Wound?

- ● 0. No _[GO TO N0415]
- ◯ 1. Yes, patient has at least one observable surgical wound
- ◯ 2. Surgical wound known but not observable due to non-removable dressing/device _[GO TO N0415]

(M1342) Status of Most Problematic Surgical Wound that is Observable

- ◯ 0. Newly epithelialized
- ◯ 1. Fully granulating
- ◯ 2. Early/partial granulation
- ◯ 3. Not healing

Skin Color

- ☑ Within Normal Limits
- ☐ Pale
- ☐ Jaundice
- ☐ Cyanotic

Skin Turgor:

- ☑ Good
- ☐ Fair
- ☐ Poor

Skin Abnormalities:

- ☑ Dry
- ☑ Warm
- ☐ Cool
- ☐ Rash
- ☐ Ulcer
- ☐ Ostomy
- ☐ Incision
- ☐ Diaphoretic
- ☑ Others (Specify): no open wounds or rash noted. Skin is intact

Additional Skin Comments

Norton Scale for Pressure Ulcer Risk Assessment

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	◯ Good	● Fair	◯ Poor	◯ Very Bad
Mental Condition	◯ Alert	◯ Apathetic	● Confused	◯ Stuporous
Activity	◯ Ambulant	● Walks With Help	◯ Chairbound	◯ Bedfast
Mobility	◯ Full	● Slightly Impaired	◯ Very Limited	◯ Immobile
Incontinence	● None	◯ Occassional	◯ Usually Urinary	◯ Urinary and Fecal
			Total Score:	15

Wound

Wound Worksheet

Fetch Wounds From Previous Wound Note

SAFETY & RISK ASSESSMENT

Home Safety Assessment

Home Environment

- ☐ Architectural Barriers
- ☑ No fire extinguisher
- ☐ No emergency exit plan
- ☐ Medications are improperly stored
- ☐ Inadequate electricity / cooling / heating
- ☐ Rodent / insect infestation
- ☐ No telephone
- ☐ Inadequate supply of water
- ☐ Unsafe electrical wiring
- ☐ Cluttered surroundings
- ☐ Inadequate sanitation / plumbing
- ☐ Nonworking stove / refrigerator
- ☐ Inadequate railing
- ☐ Inadequate smoke detectors on all levels of home
- ☐ Exposed flammables
- ☐ Unsafe oxygen use
- ☐ Unsafe storage of equipments
- ☐ Inadequate Lighting

Other:

Type of Assistance Patient Receives - other than from Home Health Agency Staff

ADL(Bathing, dressing, toileting, eating/feeding, bowel)	☐ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances)	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
Psychosocial Support	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
Assistance with doctors appointment, delivery of medications	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
Management of Finances	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations

Comments:

(M2102) Types and Sources of Assistance

Determine the ability and willingness of non-agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety (for example, due to cognitive impairment)

- ◯ 0. No assistance needed – patient is independent or does not have needs in this area
- ● 1. Non-agency caregiver(s) currently provide assistance
- ◯ 2. Non-agency caregiver(s) need training/supportive services to provide assistance
- ◯ 3. Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance
- ◯ 4. Assistance needed, but no non-agency caregiver(s) available

Safety Hazards Affecting Patient

- ☐ Stairs
- ☐ No running water, plumbing
- ☐ Inadequate lighting, heating and cooling
- ☐ Narrow or obstructed walkway
- ☐ Insect/Rodent Infestation
- ☑ Lack of fire safety devices
- ☐ No gas/electric appliance
- ☐ Cluttered/soiled living area
- ☐ No hazards identified

Others:

No fire alarms

Safety Measures

- ◯ 1 - Bleeding precautions
- ◯ 2 - O_2 precautions
- ◯ 3 - Seizure precautions
- ✓ 4 - Fall precautions
- ◯ 5 - Aspiration precautions
- ◯ 6 - Siderails up
- ◯ 7 - Elevate head of bed
- ◯ 8 - 24 hr. supervision
- ✓ 9- Clear Pathways up
- ◯ 10 - Lock w/c with transfers
- ✓ 11 - Infection control measures
- ✓ 12 - Walker/cane
- ◯ 13 - Sharps Precaution
- ✓ 14 - Other:

Covid 19 precautions
Universal precaution/911 protocol

Hospitalization Risk Assessment

Prior Pattern: Check all that apply

- ☑ > 1 Hospitalizations or ER visits in the past 12 months
- ☑ History of falls

Chronic conditions: Check all that apply (M1020/1022/1024)

- ☐ HF
- ☐ Diabetes
- ☐ COPD
- ☐ Chronic skin ulcers (Wound consult if indicated for any wounds)
- ☐ HIV/AIDS

Risk factors: (Check all that apply)

- ☐ Discharged from hospital or skilled nursing facility
- ☑ More than 2 secondary diagnoses
- ☐ Low socioeconomic status or financial concerns ♦
- ☐ Lives alone ►♦
- ☐ Inadequate support network ♦
- ☑ ADL assistance needed ►
- ☑ Home safety risks ► ♦
- ☐ Dyspnea ► ★
- ☐ Help with managing medications needed ► ★
- ☐ Non-compliance with medication regimen ♦ ★
- ☑ Confusion ♦ ★
- ☐ Pressure ulcer ★
- ☐ Stasis ulcer ★
- ☐ Overall Poor Status/Prognosis ■
- ☐ Low literacy level ♦
- ☐ Depression ♦
- ☑ ►Consider Therapy referral (PT, OT, ST)
- ☐ ♦Consider MSW referral
- ☐ ■ Consider Hospice referral
- ☐ ★Consider RN referral, if not ordered

Total number of checked boxes is:
7

Your agency may want to select a threshold score to target patients at high risk.

(For example: 5 or greater risk factors may indicate that the patient is at risk for hospitalization. Note: This number is for convenience only and has not been tested or validated. The agency may modify the score based upon the needs of their patient population.)

Carry out patient specific interventions as appropriate/ordered, if patient is at risk for hospitalization: (Coordinate with M2250)

Referrals:

- ☑ SN
- ☑ PT
- ☐ OT
- ☐ ST
- ☐ MSW
- ☐ HHA
- ☐ Dietary Consultant

Other:

Consider implementing any of the following interventions, if patient is at risk for hospitalization:

- ☑ Medication Management
- ☑ Medication Reconciliation
- ☑ Assess patient's: knowledge, ability, resources and adherence
- ☑ Education
- ☑ Patient/family education
- ☐ Enrollment into a disease management program

(Specify):

- ☐ Hospice/Palliative Referral
- ☑ Individualized Patient Emergency Care Plan
- ☑ Fall Prevention Program
- ☐ Phone Monitoring
- ☐ Telemonitoring
- ☐ Wound Care
- ☐ Front-loading Visits
- ☑ Care Coordination (physicians, hospitals, nursing homes ...)

Immunizations (M1040, M1045, M1050, M1055)

- ☐ Influenza
- ☐ Pneumococcal

Fall Prevention Program:

Other:

(M1033) Risk for Hospitalization: Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- ☐ 1. History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- ☐ 2. Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- ☐ 3. Multiple hospitalizations (2 or more) in the past 6 months
- ☐ 4. Multiple emergency department visits (2 or more) in the past 6 months
- ☐ 5. Decline in mental, emotional, or behavioral status in the past 3 months
- ☑ 6. Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- ☑ 7. Currently taking 5 or more medications
- ☐ 8. Currently reports exhaustion
- ☐ 9. Other risk(s) not listed in 1 - 8
- ☐ 10. None of the above

Additional Risk Information:

Prior Hospitalization

✓ Yes ◯ No

Number of times: 1

FALL RISK ASSESSMENT TOOL

MAHC 10* - FALL RISK ASSESSMENT TOOL

	Yes	No

Age 65+	✓	
Diagnosis (3 or more co-existing) Includes only documented medical diagnosis	✓	
Prior history of falls within 3 months An unintentional change in position resulting in coming to rest on the ground or at a lower level		✓
Incontinence Inability to make it to the bathroom or commode in timely manner. Includes frequency, urgency, and/or nocturia.		✓
Visual impairment Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tolerance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.	✓	
Impaired functional mobility May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, impaired coordination or improper use of assistive devices.		✓
Environmental hazards May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.		✓
Poly Pharmacy (4 or more prescriptions - any type) All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquilizers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.	✓	
Pain affecting level of function Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.		✓
Cognitive impairment Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsivity, memory deficits. Consider patients ability to adhere to the plan of care.	✓	
A score of 4 or more is considered at risk for falling	5	

Fire Assessment for Patient with Oxygen

☑ **Patient is not using oxygen**

Does patient have No Smoking signs posted?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient or anyone in the home smoke with oxygen in use?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are smoke detectors present and working properly?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient have a properly functioning fire extinguisher?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are oxygen cylinders stored properly?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are all electrical cords near oxygen intact and free for fraying?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient have an evacuation plan in case of fire?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are all cleaning fluids and aerosols stored away from oxygen, and not used when oxygen is in use?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient refrain from using petroleum products around oxygen?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient only used water-based body and lip moisturizers?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No

Comments:

FUNCTIONAL ASSESSMENT (SELF CARE)

(GG0100) Prior Functioning: Everyday Activities: Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.

- **3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper
- **2. Needed Some Help** – Patient needed partial assistance from another person to complete activities
- **1. Dependent** – A helper completed the activities for the patient.
- **8. Unknown**
- **9. Not Applicable**

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	✓				
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	✓				
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	✓				
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.		✓			

(GG0110) Prior Device Use

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (**Check all that apply)**

- ☐ **A. Manual wheelchair**
- ☐ **B. Motorized wheelchair and/or scooter**
- ☐ **C. Mechanical lift**
- ☐ **D. Walker**
- ☐ **E. Orthotics/Prosthetics**
- ☑ **Z. None of the above**

(GG0130) Self Care

(GG0130) SOC/ROC Performance

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.

04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.
If activity was not attempted, code reason:
07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

	A. Eating	B. Oral Hygiene	C. Toileting Hygiene	E. Shower /bathe self	F. Upper body dressing	G. Lower body dressing	H. Putting on /taking off footwear
(06) Independent	○	○	○	○	○	○	○
(05) Setup or clean-up assistance	✓	✓	✓	✓	✓	✓	✓
(04) Supervision or touching assistance	○	○	○	○	○	○	○
(03) Partial/moderate assistance	○	○	○	○	○	○	○
(02) Substantial/maximal assistance	○	○	○	○	○	○	○
(01) Dependent	○	○	○	○	○	○	○
(07) Patient refused	○	○	○	○	○	○	○
(09) Not applicable	○	○	○	○	○	○	○
(10) Not attempted due to environmental limitations	○	○	○	○	○	○	○
(88) Not attempted due to medical conditions or safety concerns	○	○	○	○	○	○	○
(-) Not assessed/no information	○	○	○	○	○	○	○

(M1800) Grooming
Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)
- ○ **0. Able to groom self unaided, with or without the use of assistive devices or adapted methods.**
- ✓ **1. Grooming utensils must be placed within reach before able to complete grooming activities.**
- ○ **2. Someone must assist the patient to groom self.**
- ○ **3. Patient depends entirely upon someone else for grooming needs**

(M1810) Ability to Dress Upper Body
Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:
- ○ **0. Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.**
- ✓ **1. Able to dress upper body without assistance if clothing is laid out or handed to the patient.**
- ○ **2. Someone must help the patient put on upper body clothing.**
- ○ **3. Patient depends entirely upon another person to dress the upper body.**

(M1820) Ability to Dress Lower Body
Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:
- ○ **0. Able to obtain, put on, and remove clothing and shoes without assistance.**
- ✓ **1. Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.**
- ○ **2. Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.**
- ○ **3. Patient depends entirely upon another person to dress lower body.**

(M1830) Bathing
Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).**
- ○ **0. Able to bathe self in shower or tub independently, including getting in and out of tub/shower.**
- ○ **1. With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.**
- ✓ **2. Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.**
- ○ **3. Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.**
- ○ **4. Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.**
- ○ **5. Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person**
- ○ **6. Unable to participate effectively in bathing and is bathed totally by another person.**

(M1840) Toilet Transferring
Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode
- ✓ **0. Able to get to and from the toilet and transfer independently with or without a device**
- ○ **1. When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.**
- ○ **2. Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).**
- ○ **3. Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently**
- ○ **4. Is totally dependent in toileting.**

(M1845) Toileting Hygiene
Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.
- ✓ **0. Able to manage toileting hygiene and clothing management without assistance.**
- ○ **1. Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.**
- ○ **2. Someone must help the patient to maintain toileting hygiene and/or adjust clothing.**
- ○ **3. Patient depends entirely upon another person to maintain toileting hygiene**

(M1850) Transferring
Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast
- ○ **0. Able to independently transfer.**
- ✓ **1. Able to transfer with minimal human assistance or with use of an assistive device.**
- ○ **2. Able to bear weight and pivot during the transfer process but unable to transfer self.**
- ○ **3. Unable to transfer self and is unable to bear weight or pivot when transferred by another person**
- ○ **4. Bedfast, unable to transfer but is able to turn and position self in bed.**
- ○ **5. Bedfast, unable to transfer and is unable to turn and position self**

(M1860) Ambulation/Locomotion
Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.
- ○ **0. Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).**
- ○ **1. With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.**

- [x] 2. Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.
- [ ] 3. Able to walk only with the supervision or assistance of another person at all times.
- [ ] 4. Chairfast, unable to ambulate but is able to wheel self independently.
- [ ] 5. Chairfast, unable to ambulate and is unable to wheel self
- [ ] 6. Bedfast, unable to ambulate or be up in a chair.

FUNCTIONAL ASSESSMENT (MOBILITY & MUSCULOSKELETAL)

(GG0170) Mobility

(GG0170A) A. Roll left and right: The ability to roll from lying on back to left and right side, and return to lying on back on the bed.

SOC/ROC Performance

- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170B) B. Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed.

SOC/ROC Performance

- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170C) C. Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.

SOC/ROC Performance

- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170D) D. Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.

SOC/ROC Performance

- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170E) E. Chair/bed-to-chair transfer: The ability to transfer to and from a bed to a chair (or wheelchair).

SOC/ROC Performance

- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170F) F. Toilet transfer: The ability to get on and off a toilet or commode.

SOC/ROC Performance

- [ ] 06. Independent
- [x] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170G) G. Car transfer: The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [x] (-) Not assessed/no information

(GG0170I) I. Walk 10 feet: Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)*

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170J) J. Walk 50 feet with two turns: Once standing, the ability to walk 50 feet and make two turns.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [x] 04. Supervision or touching assistance
- [ ] 03. Partial/moderate assistance
- [ ] 02. Substantial/maximal assistance
- [ ] 01. Dependent
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations
- [ ] 88. Not attempted due to medical conditions or safety concerns
- [ ] (-) Not assessed/no information

(GG0170K) K. Walk 150 feet: Once standing, the ability to walk at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- [ ] 06. Independent
- [ ] 05. Setup or clean-up assistance
- [ ] 04. Supervision or touching assistance
- [ ] 07. Patient refused
- [ ] 09. Not applicable
- [ ] 10. Not attempted due to environmental limitations

- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ✓ (-) Not assessed/no information

(GG0170L) L. Walking 10 feet on uneven surfaces: The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

SOC/ROC Performance

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ✓ (-) Not assessed/no information

(GG0170M) M. 1 step (curb): The ability to go up and down a curb and/or up and down one step. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ✓ (-) Not assessed/no information

(GG0170N) N. 4 steps: The ability to go up and down four steps with or without a rail. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ✓ (-) Not assessed/no information

(GG0170O) O. 12 steps: The ability to go up and down 12 steps with or without a rail.

SOC/ROC Performance

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ✓ (-) Not assessed/no information

(GG0170P) P. Picking up object: The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

SOC/ROC Performance

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ✓ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170Q) Q. Does patient use a wheelchair and/or scooter?

- ✓ 0. No _ **[GOTO M1600]**
- ◯ 1. Yes
- ◯ Not assessed (no information)

(GG0170R) R. Wheel 50 feet with two turns: Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

SOC/ROC Performance

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170RR) RR1. Indicate the type of wheelchair or scooter used.

SOC/ROC Performance

- ◯ 1. Manual
- ◯ 2. Motorized
- ◯ Not assessed (no information)

(GG0170S) S. Wheel 150 feet: Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170SS) SS1. Indicate the type of wheelchair or scooter used.

SOC/ROC Performance

- ◯ 1. Manual
- ◯ 2. Motorized
- ◯ Not assessed (no information)

Gait and Mobility Abnormalities

- ☐ No Problems Identified
- ✓ Weakness
- ✓ Poor Balance
- ☐ Bed Bound
- ☐ Chair Bound
- ☐ Joint Stiffness
- ☐ Shuffling/Wide-based Gait
- ☐ Orthopedic Aftercare
- ✓ High Risk for Falls
- ☐ Weight Bearing Restriction
- ☐ Poor Conditioning
- ☐ Fracture:
- ☐ Joints Pain:
- ☐ Amputation:
- ☐ Atrophy:

☐ Paresthesia:

☐ Foot Drop: ☐ Right ☐ Left

☐ Contractures: Site

Fall(s) since last visit (Specify):
0

Weakness (Specify):
generalized

Change in ADL (Explain):
Comments:

Range of Motion

☐ Limited Range of Motion: ☐ RUE ☐ RLE ☐ LUE ☐ LLE

☐ Quadriplegia ☐ Hemiplegia ☐ Paraplegia

Comments:

Grip Strength

☐ Normal ☑ Strong ☐ Weak ☑ Equal

☐ Unequal ☐ Bilateral ☐ Right ☐ Left

Others (Specify):

Grasp Strength

☐ Normal ☑ Strong ☐ Weak ☑ Equal ☐ Unequal

Comments:

DME and Supplies

DME Lookup

Durable Medical Equipment:
Walker

Supplies:
Non-sterile Gloves

#18B ACTIVITIES PERMITTED

- ○ 1- Complete bedrest
- ○ 2 - Bedrest/BRP
- ☑ 3 - Up as tolerated
- ○ 4 - Transfer bed/chair
- ☑ 5 - Exercises prescribed
- ○ 6 - Partial weight bearing
- ○ 7 - Independent in home
- ○ 8 - Crutches
- ○ 9 - Cane
- ○ A - Wheelchair
- ☑ B - Walker
- ○ C - No restrictions
- ○ D - Other (specify)

ENDOCRINE (DIABETIC MANAGEMENT)

Endocrine

Is patient diabetic?	○ Yes	☑ No	
Insulin dependent?	○ Yes	○ No	How Long?
Is patient independently able to draw up correct dose of insulin?	○ Yes	○ No	○ N/A
Is patient able to administer own insulin?	○ Yes	○ No	○ N/A
Is patient taking oral hypoglycemic agent?	○ Yes	○ No	○ N/A
Is patient independent with glucometer use?	○ Yes	○ No	○ N/A
Is caregiver able to draw up and administer insulin?	○ Yes	○ No	○ N/A
Is caregiver independent with glucometer use?	○ Yes	○ No	○ N/A
Does patient or caregiver routinely perform inspection of the patient's lower extremities?	○ Yes	○ No	○ N/A

Diabetic Management

☐ Diet ☐ Hypoglycemic ☐ Exercise ☐ Insulin

Insulin Administered By:

☐ N/A ☐ Caregivier ☐ Patient ☐ Skilled Nurse

S&S of Hyperglycemia

☐ Polyuria ☐ Polyphagia ☐ Polydipsia ☐ Blurred Vision ☐ Fatigue

Others (Specify):

S&S of Hypoglycemia

☐ Perspiration ☐ Dizziness ☐ Irritability ☐ Pale ☐ Shakiness

Others (Specify):

Blood glucose range: **Low to High (mg/dL):**
Comments:

PLAN OF CARE AND SKILLED INTERVENTIONS

Plan of Care Orders

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 1x/wk x 4wks

MSW Frequency:

PT Frequency: For evaluation and treatment

OT Frequency:

HHA Frequency:

ST Frequency

Dietitian Frequency:

MD SIGNATURE COVERS ALL PAGES

ADMIT PATIENT TO STAR HOME HEALTH CARE INC.
ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

The patient's homebound due to her medical status reveals she has been unable to perform basic self-care tasks independently due to her dizziness, which has progressively worsened over the past seven weeks. She has been evaluated by physical therapist, who noted her functional limitations and the need for skilled nursing and therapy to address her safety, mobility, and overall care needs. The patient has demonstrated a fall risk due to her unsteady gait and dizziness, and she requires skilled interventions to manage her medication regimen and monitor her blood pressure, particularly due to her recent initiation of antihypertensive therapy with lisinopril.

Home health services are deemed medically necessary to facilitate her recovery and improve her independence. Skilled nursing care will be required to monitor her vital signs, manage her medications, and provide education on safety precautions to mitigate fall risks. Additionally, physical therapy will be essential to help her regain her functional independence, improve her strength and balance, and assist in safely performing her ADLs. Given her current level of dependency, which includes requiring assistance with mobility and personal care tasks, as well as the need for regular assessment of her medical conditions, she qualifies for home health care services under Medicare guidelines. The anticipated length of stay for home health services is projected to be at least a few weeks, depending on her progress.

FREQUENCY AND DURATION:
HH SN: 1 time per week for 4 weeks for skilled observation and assessment, medication review and reconciliation, pain management, infection control measures, diet, home safety, fall prevention, and disease process education.
HH PT: Evaluation and treatment for mobility, gait, transfer, establish therapeutic exercise, strengthening, and fall prevention techniques training.

Teachings/Interventions

Skilled Intervention

Assessment/Instruction/Performance:
Explained Home Health service to patient/family/caregiver(s). Explained consent to care, release of liability, and patient rights & responsibilities. Consent to care, a liability release, and consent to photograph were signed by the patient/responsible party. A comprehensive assessment of all body systems with OASIS elements evaluation was initiated. Reconciled facility discharges medications against routine home prescriptions without issue. Discussed advance directives, privacy policy, COVID-19 prevention, 911 protocols, basic home safety, and fall prevention principles with patient/family/caregiver(s). Discussed the plan of care draft with the patient and explained what to expect in the next visits and the remainder of the care episode. Also explained that the initial discussion on the management of other disease processes will be done on the next visit.

A full head-to-toe assessment was completed by this writer. The patient's gait was observed; she uses a front-wheeled walker but reported that at home she often relies on furniture for support. Education was provided on the importance of using the walker instead of furniture to reduce the risk of falls. The patient's gait is unsteady, likely related to vertigo, and physical therapy is recommended. Additional education was provided on checking blood pressure prior to taking antihypertensive medications and maintaining a log to bring to appointments with her primary care provider.

SN completed assessment of neurological status, balance, and gait; patient noted to ambulate with walker with unsteady gait and lateral leaning, requiring supervision for safety. Patient demonstrated intermittent confusion and difficulty with orientation, increasing risk during position changes, patient's son is present, to continue teaching. SN instructed patient and caregiver on slow positional changes from lying to sitting and standing, allowing time between movements to reduce dizziness. Education provided on use of walker at all times during ambulation, keeping assistive device within reach, and avoiding sudden head movements that may trigger vertigo episodes. SN demonstrated safe transfer techniques from bed to chair and proper body alignment during ambulation. Caregiver instructed to remain nearby during mobility tasks and to ensure adequate lighting and clear pathways. SN encouraged sitting during episodes of dizziness and focusing on a fixed object to assist with balance recovery. Rest periods were incorporated between activities to prevent fatigue and worsening symptoms.

SN assessed patient's overall functional status and noted generalized weakness, decreased endurance, and need for assistance with ADLs. Patient appeared well-groomed but required cueing for task completion due to decreased energy and mild cognitive impairment. SN instructed patient on energy conservation techniques including pacing activities, alternating periods of activity with rest, and prioritizing essential tasks. Education provided on maintaining adequate hydration and balanced nutrition to support energy levels. SN demonstrated use of seated positions during grooming and ADLs to reduce fatigue and prevent loss of balance. Caregiver instructed on recognizing signs of increased fatigue such as slowed movements and increased confusion, and to assist as needed. SN encouraged establishment of a daily routine to improve participation and reduce exhaustion.

SN assessed pain status; patient denied pain at time of visit but has history of mild bilateral lower extremity discomfort described as dull ache. SN instructed patient on non-pharmacological pain control strategies including repositioning every 1–2 hours, elevating lower extremities when seated, and use of deep breathing exercises to promote relaxation. Education provided on use of distraction techniques such as listening to music or engaging in light conversation during discomfort. SN demonstrated proper body mechanics and positioning to reduce strain during movement and transfers. Caregiver instructed to observe for non-verbal signs of pain such as grimacing or guarding and to assist with repositioning as needed. Emphasis placed on allowing rest periods throughout the day to prevent exacerbation of discomfort.

SN performed home safety assessment noting single-level home with adequate space but need for continued fall prevention measures due to vertigo and cognitive deficits. Patient uses walker but requires reminders for consistent use. SN instructed patient and caregiver on maintaining clutter-free walkways, securing loose rugs, and ensuring floors remain clean and dry. Education provided on use of non-skid/traction socks or properly fitting footwear during ambulation. SN demonstrated safe use of walker including proper height and hand placement. Caregiver instructed to supervise ambulation and transfers, especially during episodes of dizziness or confusion. SN emphasized importance of keeping frequently used items within reach to avoid unnecessary bending or reaching. Patient encouraged to request assistance when needed. Safety strategies reinforced to reduce risk of injury and support independence within safe limits.

☑ **Response to Teaching/Interventions**

Verbalized Understanding	☑ **Patient (%):** 85%	☑ **Caregiver (%):** 90%
Return Demonstration	☐ **Patient (%):**	☐ **Caregiver (%):**
Require Further Teaching	☑ **Patient**	☑ **Caregiver**

Comments:
Son, Joseph was there during assessment and education; he verbalized 90% understanding and wrote down the information provided

Instructions during visit

☑ **Assess/Observe** ☐ **Injection Administration** ☐ **Catheter change** ☐ **IV Therapy**
☐ **Wound Care** ☑ **Teaching/Instructions**

Patient Strength

☑ **Motivated Learner** ☑ **Strong Support System** ☐ **Absence of Multiple Diagnosis**
☐ **Enhanced Socioeconomic Status**
Other:

Conclusions

☑ **Skilled Intervention Needed** ☐ **Skilled Instruction Needed** ☐ **No Skilled Service Needed**
Other:

Rehabilitation Potential

☐ **Rehabilitation potential is poor** ☐ **Rehabilitation potential is good**
☑ **Rehabilitation potential is fair** ☐ **Rehabilitation potential is excellent**
Others:

Conference With

☑ **MD** ☐ **SN** ☑ **PT** ☐ **OT**
☐ **ST** ☐ **MSW** ☐ **HHA** ☑ **DON/DOPCS**
☐ **Clinical Coordinator** ☑ **Clinical Manager**

Name:
Floresca, Aimee Cara RN, CM

Regarding:
Start of care, Plan of care, visit frequencies, and update of patient health condition.
Physician Contacted Re:
Patient's admission to Star HH, updated health condition, and visit frequencies.
Order Changes:
None

Plan for Next Visit:
Will continue to assess and evaluate the vertigo, cardiopulmonary status, and gait pain, patient understanding of the disease process, functional status, knowledge of self-care management, and overall patient health condition and reinforce teachings on the next visit.

Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge: §484.60 (2.xiii)
Initiated patient and caregiver education and training to facilitate timely discharge.

Discharge Plans

Discharge when patient is independent in management of medical needs.
Discharge when caregiver willing and able to manage all aspects of patient's care.
Discharge to care of physician.
Discharge when reliable caregiver available to assist with patient's medical needs.
When patient knows when to notify physician.
Able to stay in residence with primary caregiver's assistance or community agency's support.
When goals are met.

☐ **Written notice of discharge provided to patient**

Discharge scheduled for:
mm/dd/yyyy

Discharge Plan Discussed with:
☑ **Patient/Family** ☐ **Care Manager** ☐ **Physician** ☐ **Other (specify):**

Discharge Plan:
☑ **To self-care when goals met** ☑ **To self care when max potential achieved** ☑ **To outpatient therapy with MD approval** ☐ **Others (Specify):**

Comments:

PATIENT SUMMARY & CLINICAL NARRATIVE

Summary

The patient is a 72-year-old female recently discharged from an inpatient rehabilitation facility and admitted to home health services due to benign paroxysmal vertigo, bilateral, which significantly affects balance and increases fall risk. She is homebound as leaving the residence is medically contraindicated due to impaired balance, fatigue, and the need for human assistance and an assistive device for safe ambulation. The patient relies on furniture or walls for support and requires considerable effort to leave the home. Comorbid conditions including hypertension, hyperlipidemia, malaise, diverticulosis, and diaphragmatic hernia further contribute to decreased endurance and functional limitations, supporting the need for continued skilled nursing care.

A comprehensive assessment revealed stable vital signs with temperature 97.8°F, pulse 72 bpm, blood pressure 105/67 mmHg, and respiratory rate 16 breaths per minute. Cardiopulmonary and gastrointestinal status remain stable, and skin is intact without evidence of breakdown. Despite overall medical stability, the patient demonstrates impaired balance and requires assistance with transfers and ambulation using a walker or crutches. Cognitive assessment indicates mild impairment, with need for prompting and cueing, particularly in unfamiliar situations, as evidenced by difficulty recalling the current date. The patient remains cooperative and communicative, with no signs of depression or acute psychiatric concerns.

Functionally, the patient requires varying levels of assistance with activities of daily living, including intermittent help with bathing, dressing preparation, and hygiene support, although she is able to perform some self-care tasks independently when items are accessible. She benefits from caregiver support within the home environment and has additional assistance available from family. The patient takes multiple medications, including high-risk antiplatelet therapy, requiring continued education for safe use and adherence. Skilled nursing services are necessary to provide ongoing assessment, disease management education, fall prevention strategies, and caregiver guidance to promote safety, prevent complications, and support the patient's ability to remain in the home setting.

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Vital Signs Assessment
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Unmet
Patient's vital signs will be within normal limits.
Interventions: Assigned Staff: SN
Check VS every visit and report to PMD if patient vitals are not within the parameters.
Progress Toward Goals:
No Progress Yet
Check O2 saturation every visit for any shortness of breath on exertion, fatigue/weakness.
Progress Toward Goals:
No Progress Yet

Problem: Benign paroxysmal vertigo, bilateral
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Reduce the frequency and severity of vertigo episodes.
Interventions: Assigned Staff: SN
Educate the patient and family on strategies to manage vertigo episodes, including head positioning techniques.
Progress Toward Goals:
No Progress Yet
Monitor and document the frequency of vertigo episodes.
Progress Toward Goals:
No Progress Yet

Problem: Other malaise
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Identify and address underlying causes of malaise.
Interventions: Assigned Staff: SN
Conduct a comprehensive assessment to determine contributing factors to malaise.
Progress Toward Goals:
No Progress Yet
Encourage adequate hydration and nutrition to combat fatigue.
Progress Toward Goals:
No Progress Yet
Facilitate regular communication between the patient and healthcare team.
Progress Toward Goals:
No Progress Yet

Problem: Essential (primary) hypertension
Goals: Target Completion: 60 1 Term: Long-term Status: Unmet
Maintain blood pressure within normal limits.
Interventions: Assigned Staff: SN
Check blood pressure regularly and document findings.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on dietary modifications to lower blood pressure, such as reducing sodium intake.
Progress Toward Goals:
No Progress Yet
Review and manage medication adherence and side effects.
Progress Toward Goals:

No Progress Yet

Problem: Hyperlipidemia
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Achieve and maintain target lipid levels.
Interventions: Assigned Staff: SN
Provide nutritional counseling focused on heart-healthy diets.
Progress Toward Goals:
No Progress Yet
Assess lipid levels through regular lab tests.
Progress Toward Goals:
No Progress Yet

Problem: Diverticulitis of intestine-part unsp-w/o perf or abscess w/o bleed
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Prevent complications and promote bowel health.
Interventions: Assigned Staff: SN
Monitor bowel habits and report any changes to the healthcare team.
Progress Toward Goals:
No Progress Yet
Encourage hydration and regular physical activity as tolerated.
Progress Toward Goals:
No Progress Yet

Problem: Diaphragmatic hernia without obstruction or gangrene
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Check for any respiratory distress and manage symptoms.
Interventions: Assigned Staff: SN
Conduct respiratory assessments to check lung function and oxygenation.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on signs of respiratory distress.
Progress Toward Goals:
No Progress Yet

Problem: Rehospitalization Risk and Medication Management
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Unmet
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.
Interventions: Assigned Staff: SN
Identify Medication Interactions and evaluate patient/family/caregiver(s) understanding of medication compliance.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (03/24/2026):

Assigned Staff: Lara, Toni RN

Electronically Signed by Lara, Toni RN 03/24/2026